Dexamethasone

This monograph is under review

All doses expressed as Dexamethasone Base

FORM Oral Solution 400micrograms/ml (2mg/5ml)
(Oral solution 100microgram/ml available on request for small doses)

INDICATION

1. Bronchopulmonary Dysplasia (BPD) / Facilitating Extubation in a pre term infant
2. Cerebral Oedema
3. Treatment of post intubation laryngeal oedema

DOSE RANGE
1. BPD / facilitating Extubation in a preterm infant

LOW DOSE SCHEDULE (DART regimen)

Day of treatment (inclusive)	DOSE	FREQUENCY	ROUTE
1 to 3	60 micrograms/kg/dose	2 times daily	Oral
4 to 6	40 micrograms/kg/dose	2 times daily	Oral
7 to 8	20 micrograms/kg/dose	2 times daily	Oral
9 to 10	8 micrograms/kg/dose	2 times daily	Oral

HIGH DOSE SCHEDULE (Consultant authorisation only)

Day of treatment (inclusive)	DOSE	FREQUENCY	ROUTE
1 to 3	200 micrograms/kg/dose	3 times daily	Oral
4 to 7	200 micrograms/kg/dose	2 times daily	Oral
8 to 14	200 micrograms/kg/dose	ONCE daily	Oral

Day 15 onwards- if the response is good the course may be stopped. If the response is less than adequate, then the course should be continued at 200micrograms/kg/dose ONCE daily or on alternative days

ADDITIONAL DOSING INFORMATION

- For both dose schedules, the weight to be used to calculate the doses prescribed is to be the baby's weight at the start of therapy, unless the consultant instructs otherwise
- Regimens can be lengthened/ shortened/ repeated as clinically indicated
- The decision about the length of each section will be the responsibility of a Consultant Neonatologist/ Paediatrician
- If doses of 300micrograms/kg/day are given for more than 7 days OR doses of 150micrograms/kg/day are given for more than 14 days, LIVE vaccinations must be delayed until three months after stopping treatment as per the guidance in the green book.

2. Cerebral Oedema

Age	DOSE	FREQUENCY	ROUTE
Neonate	250microgram/kg/dose	THREE times daily for 2-3 days	Oral

3. Laryngeal oedema

Age	DOSE	FREQUENCY	ROUTE
Neonate	200microgram/kg/dose	For 3 doses only* (at 8 hourly intervals)	Oral

* Dexamethasone should be given at least four hours before attempting extubation, then four hours

after extubation. A final dose should then be administered 8 hours after second dose.

NEONATAL West of Scotland Drug Monographs
ORAL/OTHER ROUTES

CAUTIONS, CONTRA-INDICATIONS AND SIDE EFFECTS
- See Summary of Product Characteristics and most recent edition of BNF for Children

LICENSED STATUS **Not licensed for use in BPD**

APPLICABLE POLICIES West of Scotland Neonatal Guidelines
Consult local policy if applicable

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Dose may vary depending on indication, age, renal function, hepatic function, and concomitant medications.
This monograph should be used in conjunction with the terms of reference document prepared by the West of Scotland pharmacist network.
Information is correct at the time of publication and as per local practice agreement.
For further advice contact your clinical pharmacist or pharmacy department.

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